DIGITALAIKART

Premium India-First Digital Playbooks

The First Baby Playbook

India Edition

The money, benefits and paperwork system for new parents

Maternity pay | Government benefits | Hospital bills | First-year costs

VERIFIED FROM OFFICIAL SOURCES · OCTOBER 2026

A Digitalaikart Original · Not medical advice — always consult your doctor

How to use this playbook

Read it before the baby arrives, not after.

A new baby is the biggest, happiest change in your life — and one of the most expensive. The good news is that a lot of the money you will spend in the first year is not random. It is predictable. And a large part of it can be paid for, reimbursed, or avoided altogether if you know the rules and move early.

This playbook is a single, ordered system. It walks you through the money, the government benefits, the hospital bill and the paperwork of a new baby in India, in the order you will actually face them. You do not need to read it cover to cover in one sitting, but you do need to read Part 1 (the timeline) and Part 3 (the benefits you are owed) before your third trimester, because the deadlines there are real.

Every rule, amount, deadline and fee in this book has been checked against an official source — PIB, the Ministry of Health and Family Welfare, the Ministry of Women and Child Development, ESIC, UIDAI, the Registrar General of India, IRDAI, the Department of Posts and the Ministry of Labour and Employment. Those sources are listed at the back. Where a figure is an estimate rather than a fixed rule (hospital bills, for example), it is clearly shown as a range and the basis is given.

A word on what this book is — and is not

This is a money, benefits and paperwork guide. It is not medical advice, and it does not replace your doctor, your paediatrician, your hospital or a qualified financial adviser.

The vaccination schedule in Part 7 is reproduced from the Government of India's National Immunization Schedule so you know what to expect and what is free. Always follow the schedule your own doctor confirms for your child.

Government schemes, fees and interest rates change. Every figure here was verified in October 2026. Before you act on a deadline, confirm it once on the official portal named in the text.

What this playbook covers

Part	What you get
1	The first-90-days timeline — the exact order to do things, so no deadline slips
2	Your leave and pay rights: 26 weeks, the 80-day rule, ESIC, government staff
3	The government money you are owed: PMMVY, JSY, JSSK, PM-JAY and the state layer
4	The hospital bill strategy: real costs, insurance reality, the NICU risk
5	The real cost of year one — a month-by-month map and three worked budgets
6	Insurance and tax for new parents: term cover, family floater, Sections 123 and 126
7	The free vaccination schedule and U-WIN, decoded
8	Paperwork in order: birth certificate, Baal Aadhaar, passport, nominations
9	Saving and tax for your child: Sukanya Samriddhi and the education corpus
10	The 12-month action calendar
11	Returning to work: your rights, and the childcare decision maths
12	For the self-employed and informal-sector mother
13	Twins, preterm and complications: what changes
14	Worksheets and checklists you can fill in
15	A one-page quick-reference card
16	Frequently asked questions, and the full source list

A note for both parents

In most of India, the paperwork and the hospital bill land on the mother, and the income planning lands on whoever earns more. This book is written for the household, not for one parent. Where a rule applies only to women (maternity leave), it says so plainly; where it applies to both (insurance, tax, the birth certificate), it treats you as a team.

The five numbers that decide everything

If you remember nothing else from this book, remember these five. Almost every expensive mistake a new parent makes is one of them going unplanned.

The number	Why it decides so much
26 weeks	The paid maternity leave you are legally entitled to — know the 80-day rule
Rs. 5,000 / Rs. 6,000	The PMMVY benefit — claimed only if you register in time
9 months	The typical maternity-insurance waiting period — buy before the pregnancy
Rs. 20,000-1 lakh a day	The NICU rate — the biggest hidden cost around a birth
8.2%	The Sukanya Samriddhi rate — the best safe, tax-free start for a daughter

How to read this in 30 minutes

Short on time? Read Part 1 for the sequence, Part 3 for the money you are owed, Part 4 for the hospital decision, and Part 15 for the one-page summary. Come back to the rest as each stage arrives.

Contents

Twelve parts, in the order you will need them

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The three things that cost the most if you are late

1. The PMMVY registration. For the second child, you must register during the pregnancy — you cannot claim after birth. (Part 3)

2. Maternity insurance. The waiting period is usually 9 months, and up to 24-48 months on some plans. If you buy it after the test turns positive, it will not pay for this delivery. (Part 4)

3. The birth certificate. Register within 21 days and it is free and quick. Miss a year and you need a magistrate's order. (Part 8)

PART 1

The first 90 days — what to do, in order

Move in this sequence and nothing expensive falls through the cracks

Most new-parent money stress comes from doing things out of order — buying the pram before checking insurance, or filing the maternity claim after the hospital bill is already paid. This part is the master sequence. The later parts explain each step in detail.

When	Do this	Why it matters
Day 1 of knowing	Start a 'baby file' — one folder (physical or on your phone) for every document	Every claim, certificate and reimbursement will ask for the same papers. One place saves weeks.

When	Do this	Why it matters
Weeks 1-8	Register the pregnancy at the nearest Anganwadi / ASHA centre and get your MCP card	This makes you eligible for PMMVY (Rs. 5,000-6,000) and creates the official pregnancy record you will need again and again.
Weeks 1-8	Book your first antenatal check-up; keep every report	Antenatal checks are a PMMVY condition and the basis of your insurance claim.
Weeks 4-12	Open the insurance conversation — read your current health policy's maternity clause	If there is no maternity cover, a new rider bought now will not cover this delivery. Know your position early.
Weeks 8-16	Choose your delivery route: government/JSSK, insured private, or self-pay	This one decision swings your bill by Rs. 1.5-4 lakh. Decide before you are in labour.
Weeks 12-20	Recalculate term insurance and add the baby to your health floater plan	Life cover should be 15-20x annual income once a child depends on it.
Weeks 20-28	File the PMMVY first instalment (Rs. 3,000) if eligible	Do not wait for the baby. Half the money is available now.
Weeks 28-34	Collect the pre-admission paperwork your hospital will ask for	See Part 4's question list. Asking now avoids a counter argument at discharge.
Birth day	Get the hospital's birth report / discharge summary and keep the original	These are the inputs for the birth certificate, the Aadhaar and every claim.
Within 21 days	Register the birth and get the birth certificate	Free within 21 days. Every later step (Aadhaar, passport, school) depends on it.
Within 30 days	File the PMMVY second instalment after the 14-week immunisation	The final Rs. 2,000 (or Rs. 6,000 for a second girl child) is paid against the vaccination record.
Within 60 days	Apply for Baal Aadhaar, and add the baby to your insurance	The baby needs an identity for insurance, DBT and school. Birth certificate is mandatory for babies born on or after 1 Oct 2023.
Months 1-12	Run the vaccination calendar (Part 7) and the 12-month money calendar (Part 10)	Keeps you ahead of both the clinic and the cash flow.

If you only do five things

1. Register the pregnancy at an Anganwadi/ASHA centre so the PMMVY money is not lost.
2. Find out today whether your health policy actually covers maternity — and its waiting period.
3. Register the birth within 21 days; the birth certificate is free and opens every other door.
4. Raise your term life cover to 15-20x your annual income before the baby arrives.
5. Keep one 'baby file' with every report, receipt and certificate.

Twelve things people forget

- That the second instalment of PMMVY needs the 14-week immunisation done.
- That a maternity rider has a waiting period, so it must be bought before the pregnancy.
- That the hospital's package price often excludes medicines and the doctor's fee.
- That the NICU is billed separately and is the biggest hidden risk.
- That the birth certificate should be registered within 21 days, while it is free.

- That a baby needs its own passport, not an endorsement on a parent's.
- That the nursing-break right lasts until the child is 15 months old.
- That a creche is mandatory in workplaces with 50 or more employees.
- That SSY interest is calculated on the balance after the 5th of the month.
- That the term cover needs to rise the day the baby arrives.
- That childcare is the largest recurring cost of the first year.
- That one 'baby file' saves a dozen repeat trips.

PART 2

Your leave and pay rights

26 weeks of paid leave is a legal right, not a favour

India's maternity benefit is now governed by the Code on Social Security, 2020, which replaced the Maternity Benefit Act, 1961 and took effect on 21 November 2025. The headline entitlement is unchanged and generous: up to 26 weeks of paid maternity leave. What changed is where the rules live and a few of the conditions. Here is what a working mother is entitled to, and what to ask HR for.

2.1 The 26-week maternity benefit

An eligible woman gets maternity benefit equal to her average daily wages for up to 26 weeks, of which up to 8 weeks may be taken before the expected date of delivery. The rest is taken after the birth. This is paid leave — you receive your wage while you are away, and your job is protected.

Entitlement	What the law says
Paid maternity leave	Up to 26 weeks, at average daily wages; up to 8 weeks may be taken before delivery
Eligibility	Worked at least 80 days in the 12 months immediately before the expected delivery
Adoptive mother	12 weeks from the date the child is handed over (child under 3 months)
Commissioning mother	12 weeks from the date the child is handed over (surrogacy)
Work from home	Employer may allow work from home after the maternity leave, where the work allows it
Nursing breaks	Two breaks each working day until the child is 15 months old
Medical bonus	Rs. 3,500 if the employer does not provide free pre- and post-natal care
Creche	Required in establishments with 50 or more employees; four visits a day allowed

Source: PIB, Ministry of Labour and Employment (November 2025); Code on Social Security, 2020, Chapter VI.

2.2 The 80-day rule — the one condition that catches people out

To claim the paid maternity benefit you must have worked at least 80 days in the 12 months immediately before your expected delivery. If you joined a new employer recently, or took a long unpaid break, count your days now. A shortfall does not usually mean zero — ESIC rules and some state provisions can still help — but it is far better to discover this in month two than in month eight.

2.3 Government employees get more

If you or your spouse work for the Central Government, the Central Civil Services (Leave) Rules, 1972 give a richer package on top of the code:

Benefit	Central Government employee
Maternity leave	180 days, paid at the last pay drawn
Paternity leave	15 days, paid, during the wife's confinement
Child care leave	730 days over the whole service (female and single male employees)
CCL pay	100% of salary for the first 365 days, 80% for the next 365
Special maternity leave	60 days in case of stillbirth or death of a child soon after birth
Minimum CCL spell	5 days; up to 3 spells a year (6 for a single female employee)

Source: CCS (Leave) Rules, 1972, Rules 43, 43-A, 43-AA and 43-C; DoPT.

2.4 If you are covered by ESIC

Employees whose wages fall within the ESI ceiling are covered by the Employees' State Insurance Scheme. For them the benefit is paid by ESIC rather than the employer:

- 26 weeks of maternity benefit at full wage, extendable by one month on medical advice.
- Eligibility: 70 days of contribution paid in the last two contribution periods.
- A medical bonus of Rs. 15,000 where ESIC facilities are not available.
- 12 weeks for an adopting or commissioning mother.
- 6 weeks of paid leave for a miscarriage or medical termination of pregnancy.

Source: ESIC, Maternity Benefits (esic.gov.in).

2.5 Paternity leave in the private sector

Here is the honest position: there is no central statutory paid paternity leave in the private sector. Central Government employees get 15 days; some private companies offer 5-15 days as policy. If you are a father in the private sector, ask HR in writing what your company offers and when it must be applied for. Many new fathers lose the leave simply because nobody asked.

Your HR email, in one paragraph

"I am expecting a child on [date]. Please confirm in writing: (1) my maternity/paternity leave entitlement and dates, (2) whether work-from-home is available after the leave, (3) the creche facility, and (4) the documents you need from me. Please also confirm the last date to apply."

2.6 What changed in November 2025

On 21 November 2025 India brought its four labour codes into force, folding 29 old laws into four. For maternity, the practical effect is continuity with a few upgrades: the maternity provisions now sit in the Code on Social Security, 2020; proof of pregnancy or delivery can now be given by a registered medical practitioner, an ASHA, an auxiliary nurse or a midwife (not only a hospital); and the creche obligation is reinforced, with a creche allowance permitted in place of a physical facility under the 2026 Social Security (Central) Rules. The 26 weeks, the 80-day rule and the medical bonus are unchanged. If HR quotes the old Maternity Benefit Act, the answer is the same benefit under a new name.

Source: PIB, Ministry of Labour and Employment (November 2025); Social Security (Central) Rules, 2026.

2.7 How maternity pay is calculated

The benefit is paid at your average daily wages. In plain terms, take the wage you earned over a recent period, divide it by the number of days, and that daily figure is what you receive for each day of the 26 weeks. If your salary is steady, it is close to your normal monthly pay. The two things to confirm with HR are the period they average over and whether any variable pay is included — because a benefit calculated on basic pay alone is lower than one calculated on total wages.

PART 3

The government money you are owed

PMMVY, JSY, JSSK and PM-JAY — claimed, not assumed

Between the maternity benefit and the government schemes, an eligible family in India can receive real money and free care around a birth. The trouble is that none of it is automatic — every rupee needs a form, a registration and a deadline. This part covers each scheme in plain language, with the eligibility test and the traps.

3.1 PMMVY — the Rs. 5,000 / Rs. 6,000 maternity benefit

The Pradhan Mantri Matru Vandana Yojana pays cash directly to the mother's bank or post office account under the National Food Security Act, 2013. For the first living child the benefit is Rs. 5,000, paid in two instalments. For a second child, if the child is a girl, it is a further Rs. 6,000 in one instalment.

Instalment	Condition	Amount
First child — 1st	Register the pregnancy and complete at least one antenatal check-up within 6 months of your LMP date	Rs. 3,000
First child — 2nd	Childbirth is registered and the child has received the first cycle of BCG, OPV, DPT and Hepatitis-B (14 weeks)	Rs. 2,000
Second child (girl)	Registration during pregnancy, two antenatal checks, birth registered, 14-week immunisation done	Rs. 6,000

Who is eligible. Pregnant and lactating mothers from socially and economically disadvantaged households — for example, a net family income under Rs. 8 lakh a year, an e-Shram card, a BPL or NFSA ration card, an MGNREGA job card, SC/ST, or a PM-JAY beneficiary. The age band is 18 years 7 months to 55 years at the time of childbirth. Women in regular Central or State Government or PSU employment, or already receiving a similar benefit, are excluded.

The three PMMVY traps

Deadline: apply within 270 days of childbirth for the first child — but for the second child you must register during the pregnancy. Registering late loses the Rs. 6,000.

LMP date: your Last Menstrual Period date, as written on the MCP card, is mandatory. Without it the first and second instalments cannot be claimed.

Where to apply: the nearest Anganwadi worker or ASHA fills the form for you, or apply at pmmvy.wcd.gov.in. The husband's Aadhaar is not required.

Source: Ministry of Women and Child Development, PMMVY FAQs and PIB (June 2026); 4.27 crore beneficiaries paid, Rs. 20,150 crore disbursed as of 30 April 2026.

3.2 Janani Suraksha Yojana — cash for an institutional delivery

JSY pays a cash incentive to encourage delivery in a health facility. The amount depends on your state category and whether you are in a rural or urban area. It stacks on top of PMMVY — a woman can receive both.

State category	Mother (rural)	ASHA (rural)	Mother (urban)	ASHA (urban)
Low Performing States	Rs. 1,400	Rs. 600	Rs. 1,000	Rs. 400
High Performing States	Rs. 700	Rs. 600	Rs. 600	Rs. 400

Low Performing States are the ones with low institutional delivery rates (for example Uttar Pradesh, Bihar, Madhya Pradesh, Rajasthan, Assam and Jharkhand); all other states are High Performing. Eligibility is generally all women delivering in a government facility in an LPS, and BPL/SC/ST women in an HPS.

Source: National Health Mission, Janani Suraksha Yojana (nhm.gov.in).

3.3 Janani Shishu Suraksha Karyakram — the free-delivery entitlement

JSSK is the reason a government delivery can cost almost nothing. Every pregnant woman who delivers in a public health facility is entitled, free of charge, to:

- Free, cashless delivery — including a caesarean section.
- Free drugs, consumables and diagnostics.
- Free provision of blood, where required.
- Free diet during the stay — three days for a normal delivery, seven for a caesarean.
- Free transport from home to the facility, between facilities on referral, and a drop back home after 48 hours.
- Exemption from all user charges.

Sick newborns are covered too: free treatment, drugs, diagnostics, blood and transport for a sick newborn up to 30 days after birth (some states extend this to one year).

Source: National Health Mission, JSSK guidelines (nhm.gov.in).

3.4 Ayushman Bharat PM-JAY

If your household is eligible for Ayushman Bharat PM-JAY, the scheme gives up to Rs. 5 lakh a year of cashless hospital cover per family at empanelled hospitals — and delivery, including a caesarean, is covered tertiary care. If you are unsure whether you qualify, check your card status before the delivery, not at the billing counter. (Our separate Ayushman Bharat Family Playbook walks through the registration and the rejected-card fixes.)

3.5 The state layer and the ASHA

Most states run their own maternity schemes on top of the central ones — additional cash, free kits, or transport. Your ASHA (Accredited Social Health Activist) or Anganwadi worker is the single best source for what applies where you live, and they are usually the ones who file the forms. Introduce yourself early; they are the difference between knowing and guessing.

Rough value of the benefit stack for an eligible family

Maternity benefit (paid leave): your wage for up to 26 weeks.

PMMVY: Rs. 5,000 (first child) or Rs. 6,000 (second girl child).

JSY: Rs. 700-1,400 for the mother, depending on state and area.

JSSK: a delivery that could have cost Rs. 60,000-4 lakh, at close to zero in a public facility.

PM-JAY (if eligible): up to Rs. 5 lakh a year of hospital cover.

3.6 How to apply for PMMVY, step by step

1. Register the pregnancy at the nearest Anganwadi centre or with your ASHA worker, and make sure your LMP date is written on the MCP card.
2. Complete at least one antenatal check-up within six months of the LMP date.
3. Give the ASHA or Anganwadi worker your Aadhaar, MCP/RCHI card, bank or post office account details, and one eligibility document (for example a BPL/NFSA ration card, MGNREGA job card, e-Shram card or income proof under Rs. 8 lakh).
4. The worker files the form online at pmmvy.wcd.gov.in; the first instalment of Rs. 3,000 is credited by DBT.
5. After the birth, register the child and complete the first cycle of immunisation (14 weeks).
6. File the second instalment of Rs. 2,000 — the child's immunisation record is checked.
7. For a second girl child, remember the Rs. 6,000 must have been registered during the pregnancy.

3.7 Where to check your state's extra schemes

Beyond the central schemes, most states add their own maternity support — extra cash, a free delivery kit, or transport assistance. These change from year to year and state to state, so the reliable check is your state's health or women-and-child-development department, or simply your ASHA and Anganwadi worker, who usually know what is live right now. Ask once, in the first trimester, and you will not miss a window.

3.8 If your PMMVY claim is rejected

Most rejections trace back to one of four things: a missing LMP date, an ineligible or missing eligibility document, a bank account not linked to Aadhaar for DBT, or a missed deadline. The fix is usually simple — correct the detail with the same Anganwadi or ASHA worker who filed the form, re-submit, and keep the acknowledgement. Because the whole scheme runs through the local worker, the fastest route is almost always to go back to them in person rather than to chase a portal.

PART 4

The hospital bill strategy

One decision swings your bill by Rs. 1.5-4 lakh

The delivery is the single largest one-time cost of having a baby in India, and it is the cost most people plan for last. The range is enormous — from a few thousand rupees in a government hospital to several lakh in a metro corporate hospital — and the gap is not about quality of care as much as room type, city and whether a caesarean is needed. Here is what things actually cost, and how to decide.

4.1 What delivery costs, by route

Route	Normal delivery	Caesarean
Government hospital / JSSK	Rs. 5,000-25,000	Rs. 15,000-45,000
Semi-private, tier-2 city	Rs. 60,000-90,000	Rs. 1.4-2 lakh
Semi-private, metro	Rs. 78,000-1.2 lakh	Rs. 1.8-2.6 lakh
Corporate hospital, metro	Rs. 1.5-2 lakh	Rs. 3-4 lakh and above

Add prenatal care of roughly Rs. 20,000-40,000 across the nine months (consultations, scans, tests, supplements). These are indicative ranges compiled from 2026 Indian cost guides; your own hospital's

package is the only number that counts. Ask for it in writing.

Source: Livemint and CNBC-TV18 maternity cost reporting, 2026; hospital package data.

4.2 The maternity insurance reality — read this before the test

A standard health policy usually does not pay for a normal delivery. Maternity is either an add-on rider or a separate product, and it comes with three features that decide whether it is any use to you:

Feature	What to check
Waiting period	Commonly 9 months, but 24-48 months on some products. A rider bought after conception will not pay for this delivery.
Sub-limit	Maternity cover is often capped — Rs. 35,000 to Rs. 1 lakh per pregnancy is typical, well below a metro private bill.
Newborn cover	The baby is usually covered from birth up to 90 days, then must be added to the policy for an extra premium.
Corporate policy	Employer cover often excludes maternity or caps it low. Read the actual clause, not the brochure.

A useful regulatory fact: in 2022 IRDAI clarified that internal congenital anomalies are no longer treated as a 'specified disease' with a waiting period — so a newborn's internal birth condition is not excluded on that basis. External congenital anomalies may still carry a waiting period. Check your policy wording.

Source: IRDAI circular on insurance cover for new-borns (December 2022); policy documents on irdai.gov.in.

4.3 Choosing the route — the honest trade-off

Route	Best if	Watch out for
Government / JSSK / PM-JAY	You are eligible and want the lowest bill; complications are handled in referral hospitals	Crowding and privacy; confirm the facility has the specialists you may need
Insured private	You have a maternity rider past its waiting period and a hospital in network	Sub-limits below the real bill; non-payable items; the room-category clause
Self-pay private	No cover, but you want a specific hospital	Ask for a flat package price and pay-in-advance discount; get the itemised estimate

4.4 The NICU risk — the cost nobody budgets for

The single biggest financial shock around a birth is not the delivery. It is a newborn intensive care (NICU) admission, which can run Rs. 20,000 to over Rs. 1 lakh per day, and is far more likely with a premature or low-birth-weight baby. Most families badly underestimate this exposure. Two defences: make sure your policy covers the newborn from day one, and keep a dedicated buffer for exactly this. If you are delivering in a government facility, JSSK covers the sick newborn's treatment.

Source: Livemint, cost of a baby in India (2026).

4.5 The hospital question list — ask before you are in labour

- What is the all-inclusive package price for a normal delivery and for a caesarean?
- Which items are outside the package (medicines, consumables, doctor's fee, NICU)?
- Is the hospital on my insurer's cashless network, and what is the room-category limit?
- What is your NICU day rate, and is it covered?
- What documents should I bring for pre-authorisation, and who files it?

- If we pay ourselves, is there a discount for paying at admission?

The pre-authorisation rule of thumb

For a cashless claim, the hospital files the pre-authorisation request with your insurer before admission. Start it as soon as your admission date is known — not on the way to the labour room. If the insurer delays, you are entitled to escalate; keep every reference number.

4.6 Read your health policy in ten minutes

Most maternity surprises are written in the policy you already hold. In ten minutes you can find the four clauses that decide your claim:

- The maternity clause: is maternity covered at all, and is it a rider or a separate product?
- The waiting period: how many months before maternity benefits begin?
- The sub-limit: the maximum the policy pays per pregnancy — often far below the private bill.
- The newborn clause: from what day is the baby covered, and for how long (usually to 90 days)?
- The room-category clause: the room type your sum insured allows, and the co-payment if you take a higher one.

4.7 A worked cashless example

Suppose your insured private hospital quotes Rs. 1.6 lakh for a caesarean and your policy's maternity sub-limit is Rs. 75,000 with a 9-month waiting period you have already crossed. The hospital files a pre-authorisation; the insurer approves up to the sub-limit. You pay the difference — roughly Rs. 85,000 — plus any non-payable items the policy excludes. This is exactly why the sub-limit matters more than the sum insured: a Rs. 10 lakh policy can still pay only Rs. 75,000 for a delivery. Read the sub-limit before you choose the hospital, not after.

Illustrative example only; your policy's actual figures govern.

4.8 What hospitals usually bill outside the package

- Medicines and consumables used during the stay.
- The surgeon's, anaesthetist's and paediatrician's fees, which are often separate.
- The room upgrade if you take a category above your policy's limit.
- NICU or special-care nursery charges, always billed separately.
- Investigations and any blood products.

Ask for the package to be written down, with this list beside it, so the exclusions are visible before the admission rather than after.

PART 5

The real cost of year one

The first year is the most expensive per month — plan it, do not absorb it

Across India, a comfortable middle-class family spends roughly Rs. 5-10 lakh on a baby's first year when delivery is included; a lean, disciplined year can be run for Rs. 2-4 lakh, and a premium metro year can exceed Rs. 10 lakh. The monthly jump is what surprises people most: household spending typically rises 20-30% once a baby arrives. Here is where it goes.

5.1 The month-by-month cost map

Months	The big costs	How to keep it sane
1-2	Delivery, medicines, feeding support, diapers	Do not buy the luxury nursery. Babies need very little at first.
3-4	Vaccinations, paediatric visits, household help	Use the free government schedule where you can (Part 7); skip gadget purchases.
5-6	Feeding transition, a high chair, travel gear	Resist premium baby brands; borrow or buy second-hand for short-use items.
7-9	Baby-proofing, childcare arrangements, toys	Buy fewer toys than you think. Childcare is the real line item.
10-12	First birthday, travel, gifts, photos	Watch social-comparison spending — it is the biggest silent leak.

Recurring monthly costs in the first year typically include formula and baby food (Rs. 2,000-8,000), diapers (Rs. 2,500-5,000), paediatric care (Rs. 2,000-5,000) and childcare — a nanny at Rs. 15,000-25,000 a month in a metro, or a creche at Rs. 8,000-15,000. Vaccinations alone run Rs. 15,000-30,000 in the first year if you take the premium private schedule.

Source: Livemint and Coupl 2026 cost guides; CNBC-TV18 (October 2026).

5.2 Three worked budgets

Head	Lean	Comfortable	Premium
Delivery	Rs. 25,000 (govt)	Rs. 1.2 lakh (insured private)	Rs. 3.5 lakh (metro corporate)
Prenatal care	Rs. 15,000	Rs. 30,000	Rs. 60,000
Baby gear (one-time)	Rs. 15,000	Rs. 40,000	Rs. 1 lakh
First-year running costs	Rs. 1.2 lakh	Rs. 3 lakh	Rs. 6 lakh
Vaccinations	Free (UIP)	Rs. 20,000	Rs. 35,000
Indicative year-one total	Rs. 2-2.5 lakh	Rs. 5-6 lakh	Rs. 11 lakh+

These are illustrative, built from the ranges in this playbook — not quotes. Use them to set a target, then replace each line with your real number.

5.3 The baby fund — build it before the baby

Build a dedicated 'baby fund' before the baby arrives, and keep it in something you can reach within 24 hours — a savings account, a liquid fund or a short-term fixed deposit. Do not lock this money into long-term instruments; its job is liquidity, not returns.

- Dual-income household: hold 6 months of your new (higher) household expenses.
- Single-income household: hold 12 months, because if one parent stops working there is no second salary to absorb a surprise.
- Before delivery, in a metro, that usually means keeping roughly Rs. 3-4.5 lakh accessible.
- Increase your emergency fund by 30-50% the moment the baby arrives, then rebuild your regular savings.

Source: Livemint, financial planning for a baby (2026).

5.4 Ten ways to cut the bill without cutting care

1. Use the free government vaccination schedule for the core vaccines (Part 7).
2. Take the free JSSK entitlements if you deliver in a public facility.
3. Buy the pram, crib and high chair second-hand — they are used for months, not years.
4. Ask the hospital for a flat package price and a pay-at-admission discount.
5. File every insurance claim, however small — it adds up.
6. Buy fewer toys; rotate a small set instead.
7. Use the creche at work if your employer has one (Part 2).
8. Skip the education SIP in month two — stabilise your cash first, then invest.
9. Resist social-comparison spending on the first birthday.
10. Claim your tax deductions (Part 6) — it is money you already earned.

5.5 The recurring-cost audit

One-time purchases are easy to see; recurring costs are what quietly drain a year. Once a quarter, list every subscription and standing baby expense — formula, diapers, a streaming plan, a baby app, a class — and ask one question of each: does this still earn its place? Cancel one per quarter and you will fund a meaningful part of the education SIP without feeling it.

Recurring cost	Typical monthly	The audit question
Diapers	Rs. 2,500-5,000	Is the brand you settled on the best value per unit?
Formula / baby food	Rs. 2,000-8,000	Bulk buying or a subscription discount?
Childcare	Rs. 8,000-25,000	Creche vs nanny vs family — reviewed at least twice a year
Baby apps / subscriptions	Rs. 200-1,000	Still used, or set-and-forgot?

5.6 Adapting your monthly budget for a baby

You do not need a new budget — you need your old one re-weighted. A simple split for a household with a new baby is: about 20% to investing, 10% to a baby reserve, 10% to an emergency reserve, the rest to essentials and guilt-free spending. The point of naming the two reserves separately is that the baby reserve funds known costs, while the emergency reserve stays untouched for the unexpected.

Share	Goes to	Why
20%	Long-term investing (SIP, PPF, SSY)	Protects the child's future even in a tight year
10%	Baby reserve	Funds the known first-year costs without dipping into savings
10%	Emergency reserve	Covers a surprise — a NICU bill, a job gap, a medical event
60%	Essentials and guilt-free spending	Rent, EMIs, food, childcare, and yes, some joy

Source: adapted from Livemint's 2026 guidance on planning for a baby.

PART 6

Insurance and tax for new parents

A child changes the size of the cover you need — not just the size of your bills

A baby does two things to your financial plan: it increases what you spend every month, and it increases what your family would need if something happened to an earner. Both have to be handled deliberately. This part is short and high-impact.

6.1 Recalibrate your term life cover

Term insurance is the cheapest way to protect a child. The working rule is cover of 15-20 times your annual income, with the policy running until your youngest child is financially independent. Two adults, two policies. Avoid mixing insurance with investment — a pure term plan plus a separate investment does both jobs better and cheaper than a 'child plan'.

6.2 Health cover for the whole family

A family floater health policy should include newborn cover from day one — vaccinations, congenital conditions and NICU. Add the baby to the policy within the window your insurer allows (often 30-90 days after birth). If you rely on an employer policy, remember it usually ends when the job does; a personal floater is the durable one.

Cover	Target	Why
Term life	15-20x annual income per earner	Replaces your income for the child's dependent years
Family floater health	Rs. 10 lakh+ in a metro	One NICU admission can breach a smaller cover
Newborn cover	From day one, to at least 90 days	Bridges the gap until the baby is added to the policy

6.3 The tax you can claim — under the new Act's numbers

India's direct-tax law was rewritten as the Income-tax Act, 2025, and the old familiar section numbers have new ones. Two matter most to parents:

What	New Act (2025)	Old number
Health insurance premium (self, family, parents)	Section 126	80D
Savings — SSY, PPF, ELSS, life premium, tuition fees	Section 123	80C

Deductions under these sections apply only if you are on the old tax regime. If you have chosen the new regime, the standard deduction and the wider slabs replace them. Run both regimes against your actual numbers before you decide — our First Salary Playbook and Income Tax Act 2025 Playbook cover this in detail.

The one-line rule for a new parent

Protect the income first (term cover), protect the health second (family floater with newborn cover), and only then optimise the tax. A deduction saves you a few thousand; adequate cover prevents a catastrophe.

6.4 Five money mistakes new parents make

1. Buying the education plan in month two, before the emergency fund is rebuilt.
2. Mixing insurance with investment — a 'child plan' that does neither job well.

3 Letting the maternity sub-limit go unread until the bill arrives.

4 Stopping retirement contributions entirely instead of just slowing them.

5 Skipping the birth certificate or the Aadhaar in the first weeks, then paying for it in delays.

PART 7

The free vaccination schedule and U-WIN

Know what is free, what is optional, and how to keep the record

Vaccination is the most valuable thing you will give your child in the first two years, and the core schedule is free at every government health facility in India. This part reproduces the National Immunization Schedule so you know what to expect — and explains what is free, what you might choose to pay for privately, and how the U-WIN digital record works. Follow the schedule your own doctor confirms for your child.

7.1 The National Immunization Schedule (free under the UIP)

Age	Vaccines given
At birth	BCG, Hepatitis B birth dose, OPV-0
6 weeks	OPV-1, Pentavalent-1, fIPV-1, Rotavirus-1, PCV-1
10 weeks	OPV-2, Pentavalent-2, Rotavirus-2
14 weeks	OPV-3, Pentavalent-3, fIPV-2, Rotavirus-3, PCV-2
9-12 months	Measles-Rubella-1, JE-1 (in endemic districts), PCV booster, fIPV-3
16-24 months	Measles-Rubella-2, JE-2 (endemic), DPT booster-1, OPV booster
5-6 years	DPT booster-2
10 years	Td (tetanus-diphtheria)
16 years	Td

Pregnant women also receive Td-1 and Td-2 (or a single Td booster if already vaccinated within three years). The programme reaches about 2.9 crore pregnant women and 2.54 crore newborns every year, free of cost, and full immunisation coverage reached 98.4% in January 2026.

Source: MoHFW National Immunization Schedule and PIB press note (March 2026).

7.2 Free versus private — what you might pay for

The vaccines above are free at government facilities. Private paediatricians often offer the same vaccines plus optional ones not in the free schedule — Rotavirus and PCV are now free in many states; others such as Varicella, Hepatitis A and Meningococcal may be extra. A full premium private schedule can add Rs. 15,000-30,000 over the first two years. There is no harm in using a mix: take the free core vaccines at a government centre and any optional ones privately. Decide in advance which you are doing where, so you are not surprised at the counter.

7.3 U-WIN — the digital vaccination record

U-WIN is the government's digital platform for vaccination records, alongside the ABHA health ID. Registration is free, and you can book a session at a government facility 'anytime, anywhere', search the nearest centre and keep a permanent digital record of your child's doses. This record is also what the PMMVY second instalment is checked against, so keep it current.

Source: U-WIN (uwin.mohfw.gov.in), MoHFW.

7.4 HPV — the free vaccine for girls

In February 2026 India launched a nationwide free HPV vaccination campaign, targeting roughly 1.2 crore girls around 14 years of age, to protect against cervical cancer. The single-dose vaccine is given free at government health facilities and continues to be available on routine immunisation days. Parental consent is required.

Source: PIB, nationwide HPV vaccination campaign (March 2026).

7.5 When to call the doctor

Vaccines prevent disease; they do not replace judgment. For a newborn, seek medical care promptly for a fever, poor feeding, unusual drowsiness, breathing difficulty, persistent vomiting or a change in the colour of the skin or lips. Keep your paediatrician's number and your nearest hospital's emergency number in the baby file. This book does not diagnose — when in doubt, call.

7.6 The optional private vaccines

Beyond the free schedule, a paediatrician may offer additional vaccines. These are choices, not obligations — discuss the timing and the need with your doctor. A rough guide to what parents commonly pay for privately:

Optional vaccine	Protects against	Note
Hepatitis A	A liver infection	Two doses, often after the first year
Varicella (chickenpox)	Chickenpox	Two doses
Influenza (annual)	Seasonal flu	From 6 months, yearly
Meningococcal	Meningitis	As advised by your paediatrician
Typhoid conjugate	Typhoid	Often after 6 months

This is a reference list, not medical advice. Your paediatrician decides what your child needs and when.

7.7 A vaccination-day checklist

- Carry the MCP card and the U-WIN record.
- Note the vaccine names and the date in your own tracker (Part 14).
- Ask the nurse which vaccines were given and when the next dose is due.
- Keep the child at the centre for the recommended observation period after the dose.
- Note any reaction and when to call the doctor, as advised at the centre.

PART 8

Paperwork, in order

The birth certificate opens every other door — get it in the first three weeks

The administrative side of a new baby is a short, ordered list. Do it in this sequence and every later step is easy. Do it out of sequence and you will be making repeat trips.

8.1 The birth certificate — free within 21 days

Registering a birth is mandatory under the Registration of Births and Deaths Act, 1969. Institutional births are registered by the hospital; for a home birth, a family member must report it. The fees are small but rise sharply with delay:

Reported within	Fee	What is required
21 days	Free	Standard birth report (usually filed by the hospital)
22 days to 1 month	Rs. 2	Late fee
1 month to 1 year	Rs. 5	Late fee plus a declaration and permission
After 1 year	Rs. 10	Magistrate's order plus documents

The registrar issues the certificate free of charge, normally within seven days of registration. Many states let you apply and download it online through the Civil Registration System portal (crsorgi.gov.in). If the certificate is issued without the child's name, you can add the name later following the prescribed procedure.

Source: RBD Act, 1969, Sections 12 and 13; Municipal Corporation of Delhi and Maharashtra birth-registration fee schedules; crsorgi.gov.in.

8.2 Baal Aadhaar — the child's first identity

Aadhaar enrolment is free for any age, including a newborn. For a child under five, only a photograph is captured (fingerprints and iris are not mature), and one parent authenticates on the child's behalf. The parent must already have Aadhaar. A birth certificate is mandatory for any child born on or after 1 October 2023. A child's Aadhaar is normally generated within about 30 days. Two mandatory biometric updates follow — at age 5 and at age 15 — and both are free when done in the 5-7 and 15-17 windows.

Source: UIDAI, enrolment of children and UIDAI's waiver of biometric-update charges (October 2025).

8.3 Passport for a minor

A child can no longer be endorsed on a parent's passport — a separate passport is required. The validity of a minor's passport is five years or until the child turns 18, whichever is earlier (15-18 year olds may opt for a full 10-year passport). A birth certificate is the proof of date of birth. Both parents' consent is normally needed, signed on Annexure D at the Passport Seva Kendra; where one parent's consent is truly unavailable, a declaration on Annexure C is used. If either parent holds a valid passport with the spouse's name endorsed, the minor's application can usually avoid police verification.

Source: Passport Seva (passportindia.gov.in), FAQs on minor passports.

8.4 Update nominations and the boring-but-vital list

- Add the baby to your health policy and update nominations on EPF, insurance and demat accounts.
- Register the child on your employer's records for any childcare or education allowance.
- Keep certified copies of the birth certificate in the baby file — you will need several.
- Write or update a simple will once you have a child; it prevents years of legal trouble.

8.5 The documents vault

Keep one physical folder and one digital backup (a cloud folder or a photos album) with these, because every claim, admission and application will ask for some of them:

- MCP card and every antenatal report.
- Hospital birth report, discharge summary and the itemised bill.
- Birth certificate (and a few certified copies).
- Baal Aadhaar and the parents' Aadhaar.
- Insurance policy, card and every pre-authorisation reference number.

- Vaccination record (U-WIN) and the paediatrician's notes.
- PMMVY, JSY and any other benefit acknowledgements.

8.6 Correcting or adding a name on the birth certificate

Two common fixes. If the certificate was issued without the child's name, you can usually add it within a year of registration through the normal application; after that, a small late fee and the prescribed procedure apply. For a spelling or detail correction, apply to the registrar with a supporting document. Handle both early — schools, passports and scholarships all read this document, and a mismatch causes avoidable delays later.

PART 9

Saving and tax for your child

A small, boring, early start beats a large, late scramble

There are two separate jobs here: building a long-term education corpus, and — if you have a daughter — using the best small-savings scheme in the country. Do not confuse them with insurance. Buy protection as protection; invest for growth as investment.

9.1 Sukanya Samriddhi Yojana — the girl-child scheme

SSY is a government-backed account for a girl child under the age of 10. It pays the highest rate among all small-savings schemes — 8.2% a year, unchanged for the October-December 2026 quarter — and the interest is compounded annually. Deposits are made for 15 years; the account matures 21 years after opening. Up to 50% can be withdrawn after the child turns 18 for higher education, and the account can be closed early for her marriage after 18. The income is exempt, and the deposit qualifies for deduction under the Income-tax Act, 2025 (Section 123).

SSY — the essentials	Detail
Interest rate (Oct-Dec 2026)	8.2% a year, compounded annually
Minimum deposit	Rs. 250 a year
Maximum deposit	Rs. 1.5 lakh a year
Eligibility	Girl child below 10 years; parent or guardian opens it
Deposit period	15 years
Maturity	21 years from opening
Partial withdrawal	Up to 50% after age 18, for higher education
Accounts per family	One per girl child, up to two

The maths of consistency. If you deposit the maximum Rs. 1.5 lakh a year for 15 years at 8.2%, the corpus at the 21-year maturity is roughly Rs. 71.8 lakh. A modest Rs. 1,000 a month (Rs. 12,000 a year) grows to roughly Rs. 5.7 lakh — still a meaningful head start, and proof that starting early matters more than starting big.

Source: Department of Posts / National Savings Institute; Ministry of Finance quarterly rate notification (30 September 2026); Economic Times, October 2026.

9.2 SSY vs PPF vs an equity SIP

Option	Rate / return	Best for
Sukanya Samriddhi (SSY)	8.2%, tax-free	A daughter's education or marriage, with zero risk
PPF	7.1%, tax-free	Safe, flexible long-term saving for any child
Equity index SIP	Market-linked (assume ~12% over 15-18 years)	Long-horizon education corpus where you accept market risk

For a boy, SSY is not available — use PPF plus an equity SIP. For a girl, a sensible split is SSY for the guaranteed, tax-free core and an index SIP for growth. Keep both out of your emergency fund.

9.3 The education corpus, in real numbers

Education inflation runs ahead of general inflation, so a corpus built today has to be built for tomorrow's fees. As an illustration, a Rs. 2,000 a month SIP at an assumed 12% annual return for 18 years grows to roughly Rs. 15.3 lakh; Rs. 5,000 a month grows to roughly Rs. 38.3 lakh. Start with whatever you can sustain, and increase it every year with your income.

Returns are illustrative and market-linked; they are not a promise. Discuss your actual plan with a SEBI-registered adviser.

9.4 Opening a Sukanya Samriddhi account, step by step

1. Go to a post office or an authorised bank with the girl child's birth certificate and your own KYC.
2. Fill the SSY account-opening form; the child must be below 10 years old.
3. Deposit the minimum Rs. 250 to activate the account (you can deposit up to Rs. 1.5 lakh a year).
4. Set up a standing instruction so a fixed amount is deposited each month — consistency beats size.
5. Keep depositing for 15 years; let the account run to its 21-year maturity.
6. After the child turns 18, you may withdraw up to 50% for higher education.

A note on the deposit date. SSY interest is calculated on the lowest balance between the 5th and the last day of each month, so deposit before the 5th to earn interest for the whole month. It is a small habit with a real effect over 15 years.

PART 10

The 12-month action calendar

One page to run the whole year

Month	Money	Admin and health
Before birth	Build the baby fund; check maternity insurance; raise term cover	Register pregnancy (Anganwadi/ASHA); MCP card; PMMVY instalment 1
Month 0	Confirm insurance pre-authorisation; keep the hospital estimate	Birth certificate within 21 days; collect the birth report
Month 1	Add the baby to the health policy; start the running-cost budget	Baal Aadhaar; first paediatric visit; U-WIN registration
Month 2	Claim the PMMVY instalment 2 (after 14-week immunisation); file any insurance claims	6-week vaccines; update nominations
Month 3	Open SSY (if a daughter) or PPF; set a small SIP	10-week vaccines; review the monthly budget
Month 4-6	Trim one recurring cost; increase the SIP if income allows	14-week and follow-up vaccines; childcare arrangement settled
Month 7-9	Review tax regime; collect 80D/126 documents	9-12 month vaccines; keep the baby file current
Month 10-12	Rebuild the emergency fund to 6-12 months; plan the next year	16-24 month vaccines due soon; first birthday without overspending

The first 40 days at home

The first weeks are mostly about recovery and rhythm, but a little money admin early prevents a lot of friction later. Keep it to four habits: log the running costs for two weeks so you know your real monthly number; finish the birth certificate and the Aadhaar; get the baby onto the insurance policy; and protect one hour a day of sleep for the primary caregiver, because exhausted spending is a real budget risk. Everything else can wait until the 40-day mark.

A worked case study

To see how it fits together, follow a hypothetical middle-class couple in a tier-2 city — call them the Sharmas — from the first test to the first birthday. The numbers are illustrative, built from the ranges in this playbook, and are meant to show the sequence, not to predict your bill.

Stage	What they did	What it cost or saved
Weeks 1-8	Registered the pregnancy at the Anganwadi; started the MCP card; read the health policy	Made eligible for PMMVY; discovered the policy had no maternity rider
Weeks 8-16	Chose a mid-range private hospital with a flat package; got the price in writing	Package agreed at Rs. 90,000 for a normal delivery, in advance
Weeks 20-28	Filed the PMMVY first instalment	Rs. 3,000 credited by DBT
Delivery	Normal delivery; no NICU needed	Rs. 90,000, against a metro corporate quote of Rs. 1.5-2 lakh
Within 21 days	Registered the birth; collected the certificate	Free
Within 60 days	Applied for Baal Aadhaar; added the baby to the family floater	Newborn covered from day one
Month 2	Filed PMMVY second instalment after the 14-week immunisation	Rs. 2,000 credited
Month 3	Opened SSY; started Rs. 1,000 a month; raised term cover	Long-term saving begun; family protected

Stage	What they did	What it cost or saved
Year one	Ran the free vaccination schedule; watched recurring costs	Vaccines free; first-year running cost held near Rs. 2.5 lakh

The lesson is not the specific amounts. It is that the Sharmas moved in the right order — policy first, hospital package second, benefits and paperwork alongside — and so nothing expensive surprised them.

PART 11

Returning to work after maternity leave

Your rights do not end when the leave does

The weeks before you go back to work are as financially loaded as the weeks before the birth, because this is when the childcare bill arrives. The good news is that the law gives a returning mother more than most people realise, and the childcare decision can be worked out with a simple sum rather than a feeling.

11.1 What the law guarantees on your return

Right	What it means
Same or equivalent job	Maternity leave is protected leave; your job and your service continue
Work from home	The employer may allow WFH after the leave, where the nature of the work allows it
Nursing breaks	Two breaks each working day, until the child is 15 months old
Creche access	Where the employer runs a creche (50+ employees), four visits a day are allowed
No discrimination	Wage and service decisions cannot turn on your pregnancy or maternity leave

If any of this is refused, you are not without options: raise it in writing with HR first (a dated email creates a record), then with the establishment's grievance committee, and if it is still ignored, with the labour authority for your state. The point is not to fight — it is to know that the entitlement exists before you accept a quiet 'no'.

11.2 The childcare decision, as a sum

The question 'should one of us stay home?' is really a comparison of two numbers: the lower earner's take-home pay, against the true cost of childcare plus the extra costs of both parents working (travel, help at home, convenience spending). Work it out on paper, not in your head.

Line	Example A — creche	Example B — nanny
Lower earner's take-home	Rs. 45,000 a month	Rs. 45,000 a month
Childcare cost	Rs. 12,000 (creche)	Rs. 22,000 (nanny)
Extra costs of both working	Rs. 8,000 (travel, help, food)	Rs. 8,000
Net gain from working	Rs. 25,000 a month	Rs. 15,000 a month
Reads as	Clearly worth working	Worth it, but tighter — and it hinges on job continuity

The trap is comparing gross salary to the nanny's bill. Compare take-home to the all-in cost. In many Indian metros the margin is real but not large, and a career break carries a cost the arithmetic does not show: lost increments, lost EPF and a harder re-entry. Where the numbers are close, the answer is usually to keep working if the job is stable, and to negotiate WFH or a creche rather than to resign.

Source: Maternity Benefit and Code on Social Security provisions (PIB, 2025); childcare cost ranges from 2026 Indian cost guides.

11.3 The four-week-before-return checklist

1. Confirm your return date and any WFH arrangement in writing.
2. Book the creche or finalise the nanny, and start a week early so the baby settles.
3. Restart the household routine a week before you return, not on the first day.
4. Check that your EPF and insurance contributions resumed correctly after the leave.
5. Keep the nursing-break and creche rights in mind — ask for them, they are yours.

Six myths, corrected

The myth	The reality
A normal delivery is not covered by insurance.	It is, if you hold a maternity rider or a maternity-inclusive plan and its waiting period is over.
Paternity leave is a legal right in the private sector.	There is no central statutory paid paternity leave for private employees; it is company policy.
PMMVY is only for the first child.	It also pays Rs. 6,000 for a second child if that child is a girl — but you must register during the pregnancy.
The vaccination schedule costs money.	The National Immunization Schedule is free at all government health facilities.
The baby can be added to a parent's passport.	No — a minor now needs a separate passport.
Sukanya Samriddhi is open to everyone.	It is for a girl child below 10 years; for a son, use PPF and an equity SIP.

11.4 The nursing-break and creche rights in practice

Two rights are easy to forget once you are back at work. First, you are entitled to two nursing breaks each working day until your child is 15 months old — these are in addition to your normal breaks, so plan your day around them rather than squeezing them in. Second, if your workplace has 50 or more employees it must provide a creche, and you are allowed four visits to it a day, counting your rest intervals. If your workplace instead offers a creche allowance under the 2026 rules, that is the substitute for the physical facility. Mention both to HR in writing on your first day back.

PART 12

For the self-employed and informal-sector mother

No employer, but most of the benefits still apply

If you are a freelancer, a shop owner, a domestic worker, a farm worker or a gig worker, there is no HR department to pay your maternity leave. That does not mean you are outside the system — it means you have to reach for the parts of it that do not depend on an employer, and build your own buffer for the part that does.

12.1 What still applies to you

Benefit	Applies to a self-employed / informal worker?
PMMVY (Rs. 5,000 / Rs. 6,000)	Yes — it is not tied to employment; it is tied to the household's eligibility
JSY cash assistance	Yes — for an institutional delivery, through the ASHA
JSSK free delivery	Yes — in any public health facility
PM-JAY (if eligible)	Yes — up to Rs. 5 lakh a year of hospital cover
Paid maternity leave	No — there is no employer, so this must be self-funded

12.2 Register on e-Shram

e-Shram is the national database of unorganised workers. Registering is free and it makes you reachable for social-security schemes and welfare announcements, and it counts as an eligibility route for PMMVY. It takes a few minutes at eshram.gov.in with your Aadhaar and bank details. If you work informally, register now — the schemes find registered workers, not invisible ones.

Source: e-Shram, Ministry of Labour and Employment.

12.3 Build your own maternity fund

Because no employer is paying you for 26 weeks, the self-employed mother has to fund her own leave. The rule of thumb: set aside the equivalent of three to six months of your typical income before the birth, in a liquid account. Treat it as a mandatory cost of the pregnancy, the same way you would treat a hospital deposit. A freelancer should also tell regular clients early about a planned break and, where possible, line up work for after it.

12.4 Prove your income, and cover your health

- File your income-tax return every year, even a small one — it is the document banks and insurers trust.
- Buy a personal health policy with a maternity rider well before you plan a pregnancy; there is no employer cover to fall back on.
- Keep a separate emergency fund of six months; a solo earner has no second salary to absorb a shock.
- If you are a domestic or farm worker, lean on your ASHA and Anganwadi worker — they file the JSY, JSSK and PMMVY paperwork.

PART 13

Twins, preterm and complications

The same rules, a bigger bill — plan for the tail risk

Twins and preterm births are the cases where the money plan is tested hardest, because two of the biggest costs — gear and newborn intensive care — both rise at once. The rules do not change, but the amounts do, so a plan built for one baby needs to be stress-tested.

13.1 What costs roughly double

- Baby gear: two of everything you cannot share — car seats, cots, and often prams.
- Formula and diapers: roughly double for as long as both are on the same routine.
- Childcare: a nanny for twins costs more than for one; a creche may charge per child.

- Delivery: a twin delivery is more likely to be a caesarean, and is sometimes charged higher.

13.2 The preterm risk — the one that matters most

A preterm or low-birth-weight baby is far more likely to need NICU care, and NICU is the single largest cost around a birth — roughly Rs. 20,000 to over Rs. 1 lakh per day. If a twin pregnancy is likely, do two things early: confirm that your policy covers the newborn from day one to at least 90 days, and set aside an explicit NICU buffer on top of the baby fund. In a public facility, JSSK covers the treatment of a sick newborn.

13.3 The scheme fine print for twins

PMMVY pays for the first two living children, and for a second child only if that child is a girl. The scheme's own guidance confirms that if a beneficiary delivers twins or triplets in the second pregnancy and one or more of the children is a girl, the girl-child incentive applies. Because the amounts and conditions are specific, confirm your case with the Anganwadi worker who files the form rather than assuming.

Source: Ministry of Women and Child Development, PMMVY FAQs (twins/triplets guidance).

If a complication is likely, do this now

Get the NICU day rate and its insurance status from your hospital in writing.

Increase the accessible buffer — aim for the baby fund plus a NICU reserve.

Confirm the newborn-cover start date on your policy (ideally day one).

Keep the birth certificate and every discharge summary — complications generate more paperwork, not less.

PART 14

Worksheets and checklists

Print these, or copy them into a note on your phone

14.1 The baby fund worksheet

Line	Your number
Monthly household expenses today	
Expected monthly expenses after the baby	
Months of cover you want (6 or 12)	
Baby fund target (months x new monthly expenses)	
Delivery package price (from your hospital)	
NICU reserve you are keeping	
Where the money is kept (must be reachable in 24 hours)	

14.2 The paperwork checklist

Done	Item
[ ]	Pregnancy registered at Anganwadi / ASHA; MCP card obtained
[ ]	PMMVY instalment 1 filed
[ ]	Maternity insurance clause read and understood
[ ]	Delivery route decided; hospital package price in writing
[ ]	Birth certificate registered (within 21 days)
[ ]	PMMVY instalment 2 filed after the 14-week immunisation
[ ]	Baal Aadhaar applied for
[ ]	Baby added to the health policy; nominations updated
[ ]	Sukanya Samriddhi or PPF opened; SIP started
[ ]	Term cover raised to 15-20x income
[ ]	Baby file complete and stored in one place

14.3 The hospital question sheet

Question	Answer / notes
All-inclusive package: normal delivery?	
All-inclusive package: caesarean?	
Items outside the package?	
On my insurer's cashless network? Room limit?	
NICU day rate, and is it covered?	
Pre-authorisation: who files it, and when?	
Discount for paying at admission?	

14.4 The vaccination tracker

Age	Due on	Given on
At birth		
6 weeks		
10 weeks		
14 weeks		
9-12 months		
16-24 months		

Glossary

Term	Plain meaning
MCP card	Mother and Child Protection card — your official pregnancy and child health record
LMP date	Last Menstrual Period date; used to date the pregnancy and claim PMMVY
ANC	Ante-natal check-up during pregnancy
UIP	Universal Immunisation Programme — the free national vaccination programme
U-WIN	The government's digital platform for vaccination records
DBT	Direct Benefit Transfer — money paid straight to your bank account
NICU	Newborn Intensive Care Unit — for sick or preterm babies
Pre-authorisation	The insurer's approval, sought before admission, for a cashless claim
PMMVY	Pradhan Mantri Matru Vandana Yojana — the maternity cash benefit
JSSK / JSY	Free-delivery entitlements / cash incentive for institutional delivery

PART 15

The quick-reference card

The whole book on one page

Item	The number / rule
Maternity leave (private sector)	Up to 26 weeks paid; 80 days worked in 12 months to qualify
Government employees	180 days maternity; 15 days paternity; 730 days child care leave
ESIC	26 weeks at full wage; Rs. 15,000 medical bonus
PMMVY	Rs. 5,000 first child; Rs. 6,000 second girl child; apply within 270 days
JSY	Rs. 700-1,400 to the mother, depending on state and area
JSSK	Free delivery, C-section, drugs, diagnostics, diet and transport in public facilities
PM-JAY	Up to Rs. 5 lakh a year of cashless hospital cover, if eligible
Delivery cost	Rs. 5,000 (govt) to Rs. 4 lakh (metro corporate C-section)
First year of baby care	Rs. 80,000 to Rs. 3 lakh, depending on city and choices
Maternity insurance	Waiting period usually 9 months; sub-limit often Rs. 35,000-1 lakh
Newborn insurance cover	Usually from birth to 90 days, then add to the policy
Birth certificate	Free within 21 days; then Rs. 2, Rs. 5, then Rs. 10 plus an order
Baal Aadhaar	Free; birth certificate mandatory for babies born on or after 1 Oct 2023
Minor passport	Separate passport; valid 5 years or until age 18
Sukanya Samriddhi	8.2% (Oct-Dec 2026); Rs. 250 to Rs. 1.5 lakh a year; matures in 21 years
Term cover	15-20x annual income, per earning parent

PART 16

Frequently asked questions

The things new parents ask most

Is the 26-week maternity leave for every working woman?

It applies to eligible women covered by the Code on Social Security, 2020 — broadly, those who have worked at least 80 days in the 12 months before the expected delivery. Central Government employees get 180 days under their own leave rules, and ESIC-insured women get 26 weeks at full wage through ESIC.

Do I get paid during maternity leave?

Yes. Maternity benefit is paid at your average daily wages for up to 26 weeks. It is paid leave, and your job is protected.

Can I claim PMMVY and JSY together?

Yes. JSY is paid in addition to PMMVY; the two are designed to stack.

I missed the PMMVY registration — is it too late?

For the first child you can apply within 270 days of childbirth. For the second girl child you must have registered during the pregnancy, so timing is critical.

Does my normal health insurance cover delivery?

Usually not without a maternity rider or a maternity-inclusive plan, and those carry a waiting period — commonly 9 months, sometimes up to 48. A rider bought after conception will not cover that delivery.

When should I add the baby to my health policy?

Within the window your insurer allows — often 30-90 days after birth. The baby is usually covered from birth up to 90 days even before you add them.

Is the government vaccination schedule really free?

Yes. The National Immunization Schedule vaccines are free at all government health facilities. Private clinics may charge for the same vaccines plus optional ones.

How soon must I register the birth?

Within 21 days it is free and simple. Delays add fees, and after one year you need a magistrate's order.

Is a birth certificate needed for Aadhaar?

Yes — for any child born on or after 1 October 2023, the birth certificate is mandatory for Aadhaar enrolment.

Is Sukanya Samriddhi only for daughters?

Yes. For a son, use PPF plus an equity SIP instead.

Should I stop my retirement savings to fund the baby?

No. Adjust the pace, not the direction. A pause now costs far more later, because of compounding. Use the baby fund for the short term and keep long-term investing running.

What is the biggest hidden cost?

A newborn NICU admission, at Rs. 20,000 to over Rs. 1 lakh a day. Insure for it and keep a buffer.

Where to get help

Save these numbers in the baby file and in both parents' phones. They are national and free.

Number	What it is for
112	All emergencies — police, fire, ambulance
104	National health helpline (medical advice and guidance)

Number	What it is for
181	Women's helpline (safety, support, complaints)
1098	Childline — for any child in need of care and protection

For scheme questions, the fastest route is usually in person: your ASHA or Anganwadi worker for PMMVY, JSY and JSSK, and your hospital's insurance desk for a cashless claim. For a denied or delayed health-insurance claim, escalate in writing and keep every reference number.

Sources and further reading

Every figure in this playbook was checked against the official or primary source named below, in October 2026. Where a range is given (hospital bills, for example), it is an indicative estimate and the basis is stated.

- Ministry of Labour and Employment / PIB — Code on Social Security, 2020 and the labour codes effective 21 November 2025 (maternity benefit, creche, work from home, medical bonus).
- Department of Personnel and Training — CCS (Leave) Rules, 1972, Rules 43, 43-A, 43-AA and 43-C (maternity, paternity and child care leave).
- ESIC — Maternity Benefits (esic.gov.in): 26 weeks, Rs. 15,000 medical bonus, 70-day eligibility.
- Ministry of Women and Child Development — PMMVY FAQs and PIB releases (Rs. 5,000 / Rs. 6,000; instalments; 270-day window; eligibility).
- National Health Mission — Janani Suraksha Yojana and Janani Shishu Suraksha Karyakram guidelines (nhm.gov.in).
- National Health Authority — Ayushman Bharat PM-JAY (Rs. 5 lakh cover).
- Ministry of Health and Family Welfare — National Immunization Schedule; U-WIN (uwin.mohfw.gov.in); PIB on the HPV campaign and indigenous Td vaccine (2026).
- Registrar General of India — Registration of Births and Deaths Act, 1969 (Sections 12 and 13) and the Civil Registration System (crsorgi.gov.in); state fee schedules.
- UIDAI — enrolment of children below five, the birth-certificate requirement and biometric-update rules.
- Passport Seva / Ministry of External Affairs — FAQs on minor passports (passportindia.gov.in).
- Department of Posts and National Savings Institute — Sukanya Samriddhi Account scheme; Ministry of Finance quarterly rate notification (30 September 2026).
- IRDAI — circular on insurance cover for new-borns (December 2022).
- Livemint and CNBC-TV18 — 2026 reporting on the cost of a baby and maternity insurance in India (indicative cost ranges).

Disclaimer

This playbook is general information about money, government benefits and paperwork for new parents in India. It is not medical, legal, tax or investment advice, and it does not replace your doctor, your hospital, a lawyer or a qualified financial adviser.

Scheme rules, fees and interest rates change. Confirm the current position on the official portal named in the text before you rely on a deadline. Where a decision is personal and consequential, take advice from a licensed professional.

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